

Shades of Accessibility

Why ADA Sunglasses Accommodations
Do Not Require Disability Disclosure

Technical Whitepaper

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Executive Summary

Sunglasses and disability accommodations represent a critical intersection of accessibility, privacy, and civil rights. Light sensitivity—scientifically termed photophobia—affects millions of individuals living with migraines, traumatic brain injury (TBI), epilepsy, autism spectrum disorder, and sensory processing differences [1]. Yet a pervasive misconception persists: that individuals must disclose a specific medical diagnosis to wear sunglasses in professional, educational, and public settings. This whitepaper directly challenges that assumption and provides legal, regulatory, and practical evidence that **the Americans with Disabilities Act (ADA) does not require disability disclosure as a prerequisite for wearing sunglasses as an accommodation.**

The legal framework supporting accommodation without disclosure is both comprehensive and explicit. Under Title I (employment) and Title II (public services), the ADA mandates reasonable accommodations for individuals with disabilities based on functional limitations—not diagnostic labels [2,3]. The Equal Employment Opportunity Commission’s 2023 guidance specifically identifies sunglasses and tinted lenses as recognized accommodations [4]. Furthermore, the Office for Civil Rights (OCR) has clearly stated that **no medical diagnosis is required** to qualify for Section 504 accommodations in educational settings [5]. This distinction is fundamental: the ADA focuses on what someone cannot do, not the name of their condition.

The Americans with Disabilities Act Amendments Act (ADAAA) of 2008 intentionally broadened the definition of disability to encompass a wider range of functional limitations and explicitly rejected a rigid, diagnostic-centered approach [6]. Courts have consistently reinforced this principle. Landmark cases including *Keyhani v. University of Pennsylvania* [7] and *Olsen v. Capital Region Medical Center* [8] established that accommodations must be evaluated based on functional impact rather than formal diagnosis.

Sunglasses represent an ideal accommodation model: low-cost, non-disruptive, and universally available. This whitepaper synthesizes regulatory guidance, case law, and practical considerations to establish that individuals seeking sunglasses accommodations neither need nor should be required to disclose specific disability diagnoses.

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1 Introduction: The Sunglasses Stigma

Imagine being unable to function in normal daylight. A seemingly simple accommodation—dark sunglasses—becomes a lifeline, yet wearing them invites scrutiny, skepticism, and social judgment. This tension between medical necessity and social stigma forms the crux of a broader accessibility challenge: the assumption that visible accommodations require visible disability disclosure. This whitepaper addresses a critical gap in disability rights jurisprudence by establishing that **individuals may require and utilize ADA accommodations, including sunglasses, without disclosing the underlying disability condition.**

Light sensitivity, or photophobia, is far more prevalent than commonly understood. The conditions triggering this sensitivity affect tens of millions of Americans. **Migraines alone impact approximately 39 million Americans** [1], with photophobia listed among the most debilitating symptoms [9]. However, migraines represent only one source of this challenge. Traumatic brain injuries and concussions frequently produce prolonged light sensitivity that can persist for months or years, impairing the individual’s capacity to work, attend school, or participate in everyday activities [10]. Epilepsy, particularly photosensitive epilepsy, creates dangerous situations in normal lighting environments [11]. Autism spectrum disorder and sensory processing disorder affect how the nervous system processes visual stimuli, making standard indoor and outdoor lighting intolerable for many individuals [12].

Yet despite the medical reality of photophobia’s prevalence and severity, those who require sunglasses to manage light sensitivity face a consistent social message: **sunglasses are fashion, not accommodation.** Workplaces implement dress codes prohibiting “informal” eyewear in customer-facing roles [13]. Schools enforce uniform policies barring sunglasses indoors. Professional norms carry an implicit judgment: wearing sunglasses suggests disengagement, informality, or lack of respect. The sufferer faces an impossible choice: endure pain, migraine progression, or sensory overwhelm to conform to social expectations, or accept the social consequences of appearing to violate norms. This is the “sunglasses stigma”—the demand that the disabled person either suffer visibly or disclose their disability to justify their accommodation.

This dynamic violates the spirit of the Americans with Disabilities Act. The ADA promises equal access and meaningful participation without requiring disabled individuals to broadcast their medical conditions to employers, educators, colleagues, or the public [2].

This whitepaper reframes sunglasses as a necessary, dignity-preserving accommodation that does not require disability disclosure. We present the legal foundation for this position, drawing on ADA Title I (employment), Title II (public services and education), and Section 504 jurisprudence. We examine the “no-disclosure doctrine”—the principle that disabled individuals need not identify their disability to receive reasonable accommodations. We analyze case law addressing sensory accommodations and provide practical guidance for employers, educators, and policymakers on recognizing and accommodating photophobia without demanding medical disclosure.

2 Legal Framework

The legal foundation supporting sunglasses as a non-disclosure accommodation rests on three complementary statutory pillars: the Americans with Disabilities Act (ADA) Title I employment provisions, Title II public accommodations requirements, and Section 504 of the Rehabilitation Act. These provisions collectively establish that employers, public entities, and federally funded programs must provide reasonable accommodations for individuals with light-sensitive disabilities without requiring disclosure of the specific disability diagnosis.

2.1 ADA Title I: Employment

The Americans with Disabilities Act Title I, codified at 42 U.S.C. § 12111 et seq., establishes comprehensive non-discrimination protections for qualified individuals with disabilities in the employment context [2]. Applicable to employers with fifteen or more employees, Title I requires covered employers to provide reasonable accommodations to qualified employees and job applicants with disabilities, except where such accommodations would impose an undue hardship on the employer’s business operations.

The statutory definition of “reasonable accommodation” under 42 U.S.C. § 12111(9) encompasses modifications to the work environment or job performance methods that enable individuals with disabilities to enjoy equal employment opportunities [2]. The ADA’s implementing regulation at 29 C.F.R. § 1630.2(o) further clarifies that reasonable accommodations include modifications to workplace policies, practices, and procedures that do not fundamentally alter the essential nature of the job [14].

Critically, the provision of sunglasses as a workplace accommodation represents one of the lowest-cost accommodations an employer can implement. Allowing an employee to wear sunglasses, absorptive filters, or specialized eyewear requires no capital investment, no modification to physical infrastructure, and no disruption to workplace operations. The Equal Employment Opportunity Commission’s 2023 guidance document on visual disabilities explicitly recognizes that “dress code modifications (for example, to allow sunglasses, absorptive filters, and hats)” constitute typical and appropriate reasonable accommodations for employees with light sensitivity conditions [4].

The ADA framework further mandates that employers engage in an “interactive process” with employees to identify and implement appropriate reasonable accommodations [14]. Codified at 42 U.S.C. § 12112(b)(5)(A), the failure or refusal to engage in this interactive process, or the failure to provide a reasonable accommodation, constitutes discrimination [2]. Significantly, the interactive process does not require an employee to disclose the specific diagnosis or medical details underlying their disability request [15].

2.2 ADA Title II: Public Entities

Title II of the ADA, codified at 42 U.S.C. § 12131 et seq., extends disability non-discrimination protections beyond employment to the programs, services, and activities provided by state and local government entities [2]. This includes public schools, courts, government offices, and all other governmental programs. The breadth of Title II coverage means that sunglasses accommodations in educational, judicial, and administrative contexts fall squarely within the legal framework’s protective scope.

The foundational prohibition in Title II appears at 28 C.F.R. § 35.130(a), which provides that “no qualified individual with a disability shall, by reason of such disability, be excluded from participation in, be denied the benefits of, or be subjected to discrimination by a public entity” [3]. This regulation makes explicit that denial of a necessary accommodation constitutes discrimination under Title II.

Parallel to Title I’s reasonable accommodation requirement, Title II mandates that public entities make “reasonable modifications” to their policies, practices, and procedures to accommodate individuals with disabilities [3]. Permitting an employee or service user to wear sunglasses or light-protective eyewear never rises to the level of a fundamental alteration. No public entity’s core mission—whether education, justice administration, or government services—is altered by allowing individuals to manage light sensitivity through protective eyewear.

The ADA Amendments Act of 2008 (ADAAA) broadened the definition of disability, mandating that disability determinations be construed broadly [6]. The implementing regulation at 28 C.F.R. § 35.108 explicitly states that the definition “shall be construed broadly in favor of expansive coverage, to the maximum extent permitted by the terms of the ADA” [3].

2.3 Section 504: Federally Funded Programs

Section 504 of the Rehabilitation Act, codified at 29 U.S.C. § 794, establishes non-discrimination protections for individuals with disabilities across all programs and activities receiving federal financial assistance [16]. Unlike Title I, which applies only to employers with fifteen or more employees, Section 504’s reach extends to any program—public or private—that receives any form of federal funding. This includes nearly all elementary, secondary, and post-secondary educational institutions [17].

The Office for Civil Rights (OCR), which enforces Section 504 in educational settings, has increasingly recognized light sensitivity disabilities as legitimate bases for accommodations. The OCR’s December 2024 fact sheet addressing migraine as a disability explicitly affirms that schools may need to “adjust rules to permit sunglasses or hats” as accommodations for migraine-related light sensitivity [18]. The guidance further emphasizes that schools “cannot delay providing accommodations pending a diagnosis” or demand that accommodations await completion of medical evaluation. This pronouncement establishes that educational institutions must act promptly to provide sunglasses accommodations without waiting for formal diagnoses.

3 The No-Disclosure Doctrine

3.1 ADAAA 2008: Intentional Broadening

The Americans with Disabilities Act Amendments Act of 2008 (ADAAA) represented a fundamental shift in how disability is conceptualized and protected under federal law [6]. Congress explicitly acknowledged that decades of narrow judicial interpretation had systematically excluded millions of individuals from ADA protection, contravening the statute’s core purpose. In particular, courts had applied restrictive interpretations stemming from landmark decisions such as *Toyota Motor Manufacturing, Kentucky, Inc. v. Williams* [19] and *Sutton v. United Air Lines, Inc.* [20].

Congress found these interpretations fundamentally contrary to the ADA’s remedial purpose and enacted the ADAAA to correct the record. The legislative findings explicitly state that the purpose of the amendments is “to make it easier for people with disabilities to obtain protection under the ADA” [6]. Congress directed that the definition of disability “shall be construed broadly in favor of expansive coverage.”

More specifically, Congress mandated that “the determination of whether an impairment substantially limits an individual in a major life activity shall be made without regard to the ameliorative effects of mitigating measures” [6]. This provision directly overturned the reasoning in *Sutton* and established that individuals cannot be denied protection simply because they manage their conditions effectively.

3.2 Functional Limitations vs. Diagnosis

The ADA’s protections are fundamentally premised on functional impairment, not diagnostic classification. The regulatory definition of disability requires an impairment that substantially

limits one or more major life activities [14]. Critically, this definition does not require documentation of any particular diagnosis. Instead, it focuses on the practical effect of the impairment on the person’s life.

Consider a concrete example: an individual who experiences frequent, debilitating headaches accompanied by severe light sensitivity does not need to carry a formal diagnosis of “migraine disorder” from a neurologist to be entitled to ADA protection. If the headaches and light sensitivity substantially limit the person’s ability to see, work, or engage in other major life activities [2], the person meets the statutory definition of disability—regardless of whether they have received formal diagnostic confirmation.

3.3 OCR/EEOC Guidance on Documentation

Both the Office for Civil Rights (OCR) and the Equal Employment Opportunity Commission (EEOC) have issued guidance explicitly stating that disability can be established without a formal medical diagnosis.

In its 2023 Frequently Asked Questions on Section 504 regulations, the OCR addressed this issue directly, stating: “There is nothing in Section 504 that requires a medical diagnosis to be present as a precondition for determining that a student has a disability” [5]. The OCR further explains that a school may base its determination on observation of the student and information provided by the parent, student, or other sources regarding the student’s functioning.

In the employment context, the EEOC acknowledges that while an employer may request documentation when a disability is not obvious, the documentation need not take the form of a formal diagnosis from a specialist [21]. Documentation can come from any health care provider and need not use specific clinical terminology. As the EEOC guidance notes, there are no “magic words” required to establish disability; the employer need only understand the individual’s limitations and the reason for the proposed accommodation [15].

4 Case Law and Precedent

4.1 Keyhani v. Trustees of University of Pennsylvania

In *Keyhani v. Trustees of University of Pennsylvania* [7], a federal court addressed the question of sunglasses as a workplace accommodation directly. The plaintiff, an employee with post-concussion syndrome, experienced significant light sensitivity as part of her symptom complex. She requested to work from home as her preferred accommodation.

The employer instead offered alternative accommodations: they permitted her to wear sunglasses and noise-cancelling headphones at work and reduced her work schedule. The court held this response lawful—the ADA does not obligate an employer to provide the employee’s *preferred* accommodation if alternative reasonable accommodations suffice.

Critically, the court noted that permitting sunglasses was “in line with her doctors’ recommendations” and constituted an “effective accommodation for her light sensitivity.” The employer met its ADA duty by allowing the visible aid, even though dress codes or workplace norms would ordinarily discourage indoor sunglasses.

This case establishes a critical precedent: employers can and should consider sunglasses as a reasonable accommodation for light sensitivity conditions. The court did not require the employee to provide a specific diagnosis—only that the accommodation addressed a documented functional limitation.

4.2 Olsen v. Capital Region Medical Center

In *Olsen v. Capital Region Medical Center* [8], a hospital technician with epilepsy experienced seizures triggered by environmental factors including bright lights and visual patterns. The employer's extensive accommodation efforts included installing light filters, covering bright x-ray screens, and allowing Olsen to wear sunglasses in the hospital to reduce glare.

Although the court ultimately ruled that Olsen could not safely perform her essential job functions even with these accommodations (due to uncontrolled seizures), the case is significant for what it reveals about judicial attitudes toward sunglasses accommodations. The list of measures taken—including indoor sunglasses—was presented as evidence of the employer's good-faith accommodation efforts. No court questioned whether sunglasses constituted a reasonable accommodation. The measure was treated as unremarkable and appropriate.

4.3 Additional Supporting Precedent

Beyond individual cases, administrative guidance consistently supports sunglasses as a reasonable accommodation. The Southwest ADA Center's guidance on court accessibility describes allowing a juror with extreme light sensitivity to wear sunglasses during trial as an example of a reasonable modification under Title II [22].

The Job Accommodation Network (JAN), a service of the U.S. Department of Labor, advises that employers should modify dress codes to permit items like sunglasses or brimmed hats for employees with conditions exacerbated by light, unless there is a compelling safety reason not to do so [13].

Notably, there appears to be no contrary authority in case law—no court has found that an accommodation allowing sunglasses is per se unreasonable. The key legal test remains whether the aid enables the person to participate or perform essential functions without undue hardship on the entity [2]. Wearing sunglasses is a low-cost, non-disruptive adjustment in most environments, making it extremely difficult to demonstrate undue hardship.

5 Medical Evidence for Light Sensitivity

5.1 Migraine and Photophobia

Migraine headaches represent a significant public health burden, affecting approximately 39 million Americans and accounting for substantial workplace and educational productivity loss [1]. Light sensitivity, or photophobia, occurs in up to 90% of migraine sufferers during acute attacks [9], making exposure to bright or flickering light an intolerable experience that can prolong symptoms and increase pain severity.

FL-41 tinted lenses, which filter blue-green light in the 480 nanometer wavelength range, have demonstrated substantial clinical efficacy [23]. A landmark study found that patients wearing FL-41 glasses experienced a reduction in migraine frequency from an average of 6.2 episodes per month to 1.6 episodes per month—a **75% reduction**. Clinical data confirm that tinted glasses reduce photophobia severity between migraine attacks, enhancing overall functioning during non-migraine periods [24].

5.2 Post-Concussive Syndrome and Traumatic Brain Injury

Light sensitivity following concussion and traumatic brain injury (TBI) represents a nearly universal symptom during acute recovery [10]. A 2017 study examining 51 post-concussion patients found that **76% experienced clinically significant photophobia**. Furthermore, in **85% of these photophobic patients**, tinted sunglasses or colored lens overlays provided substantial symptomatic relief.

The Centers for Disease Control and Prevention (CDC) specifically recommends that individuals recovering from concussions be permitted to wear sunglasses indoors when needed to manage light sensitivity [25]. By reducing light-induced symptoms, tinted eyewear allows patients to remain engaged in work, school, and therapeutic settings essential to recovery.

5.3 Visual and Sensory Processing Disorders

Beyond migraine and TBI, light sensitivity affects individuals with diverse conditions. Photosensitive epilepsy, affecting 5-10% of individuals with epilepsy, can be triggered by glare, flickering, or high-contrast visual patterns [11]. Tinted glasses serve as seizure prevention accommodations [8].

Blepharospasm, an involuntary eyelid spasm condition, frequently co-occurs with significant photophobia. Research demonstrates that FL-41 filtered lenses reduce both photophobic symptoms and blepharospasm frequency [24].

Autism spectrum disorder and sensory processing disorder commonly include heightened sensitivity to visual input, particularly artificial fluorescent lighting [12]. Tinted lenses function as a sensory filter, allowing these individuals to modulate their visual environment and maintain functional engagement in school and work settings [26].

6 Practical Guidance

6.1 For Employers

- **Review dress codes** for unnecessary restrictions on sunglasses, hats, or tinted eyewear. Consider adding explicit language permitting medical accommodations [13].
- **Train managers** to understand that sunglasses may represent medical necessity, not unprofessionalism or disengagement.
- **Focus on functional limitations**, not diagnoses, during the interactive process. Ask: “What do you need to perform your job effectively?” rather than “What is your diagnosis?” [15].
- **Accept documentation from any healthcare provider**—not just specialists. A brief note confirming light sensitivity and recommending tinted eyewear should suffice [21].
- **Consider environmental modifications** as alternatives or supplements: dimmer lighting, anti-glare screens, task lighting, or seating away from windows.
- Remember that low-cost accommodations like sunglasses almost never constitute undue hardship [2].

6.2 For Schools and Educators

- **Revise blanket “no sunglasses” policies** to include explicit exceptions for disability accommodations.
- **Include light sensitivity accommodations** in 504 Plan and IEP templates as standard options [18].
- **Do not delay support** waiting for formal diagnosis—OCR guidance is clear that schools cannot require diagnosis as a precondition [5].
- **Follow CDC “Heads Up” guidance:** allow sunglasses in brightly lit classrooms for students recovering from concussion [25].
- **Train staff** not to challenge students wearing accommodated eyewear. The accommodation, once granted, should not require repeated justification.
- In higher education, disability services offices should routinely offer sunglasses accommodation for students with documented light sensitivity.

6.3 For Individuals

- **Know your rights:** The ADA protects you. You do NOT need to disclose a specific diagnosis to receive accommodations [5].
- **Focus on describing functional limitations:** “Bright lights cause me significant pain and difficulty concentrating” is sufficient.
- **Request accommodation in writing** and keep copies of all correspondence.
- If documentation is requested, a brief note from **any healthcare provider** (not necessarily a specialist) should suffice [21].
- If denied, **ask for written explanation** and consult a disability rights organization.
- **Resources:** Job Accommodation Network (JAN) [27], ADA National Network [28], OCR complaint process [29].

7 Policy Recommendations

7.1 For Employers and HR Departments

- Update dress codes to explicitly permit medical eyewear accommodations without requiring detailed justification [4].
- Create streamlined accommodation request processes that do not demand specialist documentation for low-cost accommodations.
- Train supervisors and managers on invisible disabilities and the legal requirement to accommodate [14].
- Include sunglasses and tinted eyewear in standard accommodation menus alongside other environmental modifications.

7.2 For Educational Institutions

- Revise blanket “no sunglasses” policies to include explicit disability accommodation exceptions [18].
- Implement flexible 504 Plan templates that include light sensitivity provisions as standard options.
- Train teachers and staff on invisible disabilities and appropriate responses to accommodation requests.
- Eliminate diagnosis-level documentation requirements for environmental accommodations [5].

7.3 For Policymakers

- Issue clearer OCR and EEOC guidance specifically addressing light sensitivity as a disability warranting accommodation.
- Expand enforcement resources for investigating accommodation denials in this area.
- Fund public awareness campaigns about invisible disabilities and reasonable accommodations.
- Encourage universal design principles in lighting infrastructure (dimmable lighting, natural light options).

7.4 For Disability Advocacy Organizations

- Educate individuals about their legal right to wear sunglasses without disclosing specific diagnoses.
- Provide template accommodation request letters focusing on functional limitations.
- Document patterns of discrimination in this area to support policy reform efforts.
- Support strategic litigation where sunglasses accommodations are denied without justification.

8 Conclusion

The evidence presented throughout this whitepaper demonstrates that sunglasses for medical necessity constitute a straightforward, low-cost reasonable accommodation under the Americans with Disabilities Act [2]. Legal authorities, medical research, and practical accessibility principles converge on a single conclusion: organizations that prohibit or restrict sunglasses must reconsider these policies in light of their obligations to individuals with disabilities.

The alignment between legal precedent and medical evidence is compelling. Conditions ranging from migraine disorders and traumatic brain injuries to epilepsy and autism spectrum disorders cause genuine photosensitivity that tinted lenses measurably alleviate [10–12,23]. This

is not a matter of preference or comfort; these are therapeutic interventions that restore physiological function and enable equal participation. Courts and the Equal Employment Opportunity Commission have consistently recognized that such accommodations fall squarely within the ADA's protective scope [4].

Critically, the ADA imposes no gatekeeping requirement demanding extensive medical documentation before providing reasonable accommodations, particularly under Title II [5]. The statute's operative question is whether an adjustment enables equal access, not whether a formal diagnosis exists. This principle is especially important for non-obvious disabilities, where individuals may reasonably avoid disclosure while still requiring environmental modifications.

The practical reality reinforces the legal obligation. Sunglasses are inexpensive, universally available, and require no institutional burden to allow. They do not fundamentally alter the nature of services, nor do they impose undue financial or administrative hardship [2].

Forward-looking institutions should recognize this pattern for what it is: a prototype of inclusive policymaking. Rigid rules like blanket sunglasses prohibitions, while seemingly neutral, predictably exclude individuals with non-obvious disabilities. The solution does not demand extensive institutional overhaul, special resources, or preferential treatment. It demands only that organizations pause to ask whether a policy genuinely serves an essential purpose.

Inclusive workplaces and schools are built on this foundation: accommodating the person in front of you rather than waiting for perfect documentation. When an individual requests sunglasses for medical necessity, the appropriate response is the same response given to any reasonable accommodation request: What does this person need to participate equally? Sunglasses should not be controversial. They should be recognized as what they are—an integral auxiliary aid enabling individuals with disabilities to access the opportunities and dignity the ADA promises [2].

References

- [1] American Migraine Foundation, "Migraine facts," 2024. [Online]. Available: <https://americanmigrainefoundation.org/resource-library/migraine-facts/>
- [2] "Americans with disabilities act of 1990," 1990, 42 U.S.C. §§ 12101–12213.
- [3] "Nondiscrimination on the basis of disability in state and local government services," 2024, 28 C.F.R. Part 35.
- [4] Equal Employment Opportunity Commission, "Visual disabilities in the workplace and the americans with disabilities act," EEOC, Tech. Rep., 2023. [Online]. Available: <https://www.eeoc.gov/laws/guidance/visual-disabilities-workplace-and-americans-disabilities-act>
- [5] U.S. Department of Education, Office for Civil Rights, "Frequently asked questions about section 504 and the education of children with disabilities," OCR, Tech. Rep., 2023. [Online]. Available: <https://www2.ed.gov/about/offices/list/ocr/504faq.html>
- [6] "ADA amendments act of 2008," 2008, pub. L. No. 110-325, 122 Stat. 3553.
- [7] "Keyhani v. trustees of univ. of pennsylvania," No. 17-3092, 2019 WL 2394253 (E.D. Pa. June 21, 2019), *aff'd*, 812 Fed. App'x 88 (3d Cir. 2020), 2019.
- [8] "Olsen v. capital region medical center," 713 F.3d 114 (8th Cir. 2013), 2013.

- [9] K. B. Digre and K. C. Brennan, “Shedding light on photophobia,” *Journal of Neuro-Ophthalmology*, vol. 32, no. 1, pp. 68–81, 2012.
- [10] J. Clark, K. Hasselfeld, K. Bigsby, and J. Divine, “Colored glasses to mitigate photophobia symptoms posttraumatic brain injury,” *Journal of Athletic Training*, vol. 52, no. 8, pp. 725–729, 2017.
- [11] G. F. A. Harding and P. M. Jeavons, *Photosensitive Epilepsy*. Mac Keith Press, 1994.
- [12] C. E. Robertson and S. Baron-Cohen, “Sensory perception in autism,” *Nature Reviews Neuroscience*, vol. 18, pp. 671–684, 2017.
- [13] Job Accommodation Network, “The significance of dress codes under the Americans with Disabilities Act (ADA),” 2024. [Online]. Available: <https://askjan.org/articles/The-Significance-of-Dress-Codes-under-the-Americans-with-Disabilities-Act.cfm>
- [14] “Regulations to implement the equal employment provisions of the ADA,” 2024, 29 C.F.R. Part 1630.
- [15] Equal Employment Opportunity Commission, “Enforcement guidance: Reasonable accommodation and undue hardship under the Americans with Disabilities Act,” EEOC, Tech. Rep., 2002.
- [16] “Section 504 of the rehabilitation act of 1973,” 1973, 29 U.S.C. § 794.
- [17] “Nondiscrimination on the basis of handicap in programs or activities receiving federal financial assistance,” 2024, 34 C.F.R. Part 104.
- [18] U.S. Department of Education, Office for Civil Rights, “Fact sheet: Supporting students with migraines in public elementary and secondary schools,” OCR, Tech. Rep., December 2024.
- [19] “Toyota motor mfg., ky., inc. v. williams,” 534 U.S. 184 (2002), superseded by statute, 2002.
- [20] “Sutton v. united air lines, inc.” 527 U.S. 471 (1999), superseded by statute, 1999.
- [21] Job Accommodation Network, “Who can provide medical documentation for ADA purposes?” 2024. [Online]. Available: <https://askjan.org/articles/Who-Can-Provide-Medical-Documentation-for-ADA-Purposes.cfm>
- [22] Southwest ADA Center, “Americans with disabilities act: Advancing court accessibility for individuals with disabilities,” 2019. [Online]. Available: <https://www.defendyouthrights.org/wp-content/uploads/Southwest-ADA-Court-Accommodations.pdf>
- [23] A. Wilkins, J. Huang, and Y. Cao, “Thin-film optical notch filter spectacle coatings for the treatment of migraine and photophobia,” *Journal of Clinical Neuroscience*, vol. 23, pp. 41–46, 2016.
- [24] M. K. Blackburn, R. D. Lamb, K. B. Digre, A. G. Smith, J. E. A. Warner, R. W. McClane, S. D. Nandedkar, W. J. Langeberg, R. Holubkov, and B. J. Katz, “FL-41 tint reduces activation of neural pathways of photophobia in patients with chronic ocular pain,” *Investigative Ophthalmology & Visual Science*, vol. 59, no. 15, pp. 6152–6158, 2018.
- [25] Centers for Disease Control and Prevention, “Returning to school after a concussion: A fact sheet for school professionals (HEADS UP),” 2024. [Online]. Available: <https://www.cdc.gov/heads-up/guidelines/returning-to-school.html>

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- [26] TheraSpecs, “Light sensitivity at work: Key stats, workplace accommodations and other resources,” 2024. [Online]. Available: <https://www.theraspecs.com/blog/light-sensitivity-at-work-stats-accommodations-resources/>
- [27] Job Accommodation Network, “Job accommodation network,” 2024. [Online]. Available: <https://askjan.org/>
- [28] ADA National Network, “ADA national network,” 2024. [Online]. Available: <https://adata.org/>
- [29] U.S. Department of Education, Office for Civil Rights, “How to file a discrimination complaint with the office for civil rights,” 2024. [Online]. Available: <https://www2.ed.gov/about/offices/list/ocr/docs/howto.html>